

SKIN & LASER

Chemical Peel Protocol

Acid selection, priming, frosting endpoints, neutralization and depth-appropriate aftercare

DOCUMENT SKN-004	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All staff performing chemical peels	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol covers superficial and medium-depth chemical peels. It defines acid selection by indication and skin type, priming requirements, the layered application method with frosting endpoints, neutralization, and aftercare by depth.

SCOPE LIMIT

Deep phenol peels are not performed in this practice. They carry cardiac arrhythmia risk requiring cardiac monitoring, and belong in a setting equipped for it. Any request for a deep peel is referred.

DEPTH IS CUMULATIVE AND IRREVERSIBLE

You can always add another layer. You cannot remove one. Every peel is approached as though it will go deeper than intended, because the variables — skin prep, degreasing, sebum, prior treatment, skin thickness — are not fully predictable.

2 Acid Selection

FRAMEWORK — CONCENTRATIONS SET BY THE MEDICAL DIRECTOR

Agent	Class	Best suited to	Notes
Glycolic acid	AHA	Photodamage, texture, dullness, fine lines.	Requires active neutralization. Time-dependent — penetration continues until neutralized.
Lactic acid	AHA	Sensitive skin, hydration, mild pigmentation.	Gentler and larger-molecule. Good entry peel for higher Fitzpatrick types.
Salicylic acid	BHA	Acne, oily skin, comedones, follicular congestion.	Lipophilic; penetrates sebum. Self-limiting. Avoid in salicylate allergy and pregnancy.
Mandelic acid	AHA	Sensitive skin, higher Fitzpatrick types, acne with pigmentation.	Large molecule, slow penetration, low PIH risk. Often the safest choice in Fitzpatrick IV–VI.
Jessner's solution	Combination	Photodamage, acne, pigmentation.	Layer-dependent depth. Contains resorcinol — screen for sensitivity.
TCA, superficial to medium	Trichloroacetic	Photodamage, pigmentation, fine wrinkling, scarring.	Self-neutralizing. Depth driven by concentration and layers. Highest risk in this kit — requires the most experience and explicit authorization.

FITZPATRICK DRIVES SELECTION

Higher Fitzpatrick types carry substantially greater post-inflammatory hyperpigmentation risk. Favour gentler agents, lower concentrations, fewer layers and more sessions rather than one aggressive treatment. Priming matters more, not less, as skin type darkens.

3 Screening & Priming

- ☐ Fitzpatrick assessment and photography per SKN-007.
- ☐ Isotretinoin within the interval set by the medical director.
- ☐ Active infection, HSV, open lesions, dermatitis or recent waxing in the field.
- ☐ HSV prophylaxis for perioral peels per the medical director where history is present.
- ☐ Keloid or hypertrophic scarring history.
- ☐ Recent laser, resurfacing, microneedling or injectable in the same area.
- ☐ Photosensitizing medications and recent sun exposure.
- ☐ Pregnancy — salicylic acid in particular is generally avoided; practice policy set by the medical director.
- ☐ Allergy screen — aspirin and salicylate for BHA; resorcinol for Jessner's.
- ☐ Realistic expectation and downtime discussion completed and documented.
- ☐ Priming regimen completed and confirmed where required.

Priming

- Priming prepares the skin, improves uniformity of penetration and reduces PIH risk. It is not optional for medium-depth peels or for Fitzpatrick III and above.
- A typical regimen uses a topical retinoid and, where authorized, a tyrosinase inhibitor for a defined period before treatment. Your medical director specifies the agents and duration.

- Retinoids are discontinued for a defined interval immediately before the peel, since they increase penetration.
- Strict sun avoidance during the priming period.
- Confirm and document compliance before treating. A patient who did not prime is rescheduled, not treated at a lower concentration as a compromise.

4 Application & Frosting Endpoints

- 1 **Set up before you begin:** neutralizer, saline, cool water, fan and gauze all within reach. Never start a peel with the neutralizer in another room.
- 2 **Protect** the eyes, nostrils and lip vermilion, and occlude with petrolatum at canthi and nasolabial folds where pooling occurs.
- 3 **Cleanse and degrease thoroughly.** Inadequate degreasing produces patchy penetration and is the main cause of uneven results.
- 4 **Apply in a consistent sequence** across defined facial zones, starting with the most resilient area — forehead — and treating thinner or more sensitive zones last.
- 5 **Time from the first application**, not from when you finish applying. Note the start time out loud.
- 6 **Observe continuously** and assess frosting between layers, not after several.
- 7 **Add layers only to reach the intended endpoint**, never to fill the remaining appointment time.
- 8 **Neutralize** at the endpoint or the maximum time, whichever comes first.
- 9 **Cool** the skin and apply the post-peel occlusive specified for the depth used.

FROSTING LEVELS

Level	Appearance	Depth	Action
Level 0	Erythema only, no frost.	Very superficial.	Acceptable endpoint for a light peel or a first treatment.
Level I	Speckled, patchy white on an erythematous background.	Superficial — epidermal.	Common target. Stop here for most superficial peels.
Level II	Uniform white frost with erythema visible through it.	Papillary dermis.	Medium-depth target. Stop. Do not add layers.
Level III	Solid, dense white frost with no background erythema.	Reticular dermis.	Too deep for this setting. Stop immediately, neutralize, cool, notify the medical director. High scarring risk.

EMERGENCY STOP

Neutralize immediately and abandon the treatment for: level III frosting, disproportionate pain, grey or dusky discoloration, blistering during application, any solution contacting the eye, or the patient becoming distressed or unable to tolerate the procedure. Eye contact requires copious irrigation and same-day ophthalmology assessment.

5 Aftercare by Depth

Depth	Expected course	Aftercare
Superficial	Mild erythema and light flaking over several days.	Bland cleanser and emollient. Broad-spectrum sun protection. No actives until settled. No picking.
Medium	Marked erythema, swelling, then sheeting desquamation over one to two weeks.	Occlusive emollient, gentle cleansing, strict sun avoidance. Daily contact for the first few days. No makeup until re-epithelialized.

- No picking, peeling or pulling loose skin — this is the most common cause of scarring after an otherwise uneventful peel. Say it more than once.
- No heat, sauna, steam or vigorous exercise until healed.
- No retinoids, acids, scrubs or exfoliation until authorized.
- Strict sun avoidance and broad-spectrum protection for an extended period; PIH risk persists well past visible healing.
- Written aftercare with warning signs and same-day contact instructions.
- Follow-up appointment scheduled before the patient leaves for medium-depth peels.

CALL THE SAME DAY FOR

Increasing rather than settling pain, spreading erythema, purulence, fever, grouped vesicles suggesting HSV, or any area that looks grey, dusky or fails to re-epithelialize on schedule. Escalate to the medical director and see the patient rather than managing by telephone.

6 Quick Reference

QUICK REFERENCE CARD

Before and during a peel

- Neutralizer, saline and water in the room** before you open the acid.
- Priming confirmed.** Not primed → reschedule, don't improvise.
- Degrease properly.** Uneven prep = uneven peel.
- Protect eyes, nostrils, lips.** Occlude the pooling points.
- Time from the first application.** Say the start time out loud.
- Assess frosting between every layer.** Level II = stop.
- Level III, dusky skin, or eye contact → neutralize and escalate.**
- Tell them not to pick.** Then tell them again.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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